

Why Your Census Number Is Not Arbitrary

What Nobody Teaches Hospice Sales Reps About the Business They Sell For

The Number You Were Given Without an Explanation

At some point early in your hospice sales career, someone handed you a goal. Maybe it was eight admissions a month. Maybe it was twelve. Maybe it was phrased as *we need to hit an ADC of 60 by Q3*.

You nodded. You went to work. And if you are honest, you never fully understood where that number came from, why it mattered so much, or what happened to the branch financially when you fell short.

That is not your fault. Nobody told you. Your sales leader was focused on hitting the number, not explaining it. The clinical team was focused on patient care, not branch economics. The executive director knew the pressure but rarely translated it into something a rep in the field could understand and care about.

This document exists to close that gap. Because when you understand why the number matters, the work changes. The urgency becomes real. The mission becomes clearer. And you become a better rep.

PART ONE

What a Hospice Branch Actually Costs to Run

Most sales reps think of their branch as a care delivery operation. It is also a business — one with a fixed cost structure that does not pause when census is low. Before a single patient is admitted, before a single referral is made, the branch is spending money every month.

Clinical Staff — The Core of the Cost Structure

The clinical team is the largest expense category a branch carries. These positions are required by Medicare Conditions of Participation and must be paid whether the branch has 20 patients or 80.

Role	Annual Salary (Approx.)
Executive Director	\$140,000
Supervisor RN Case Manager	\$110,000
RN Case Manager (x2 minimum)	\$100,000 each
Hospice Aide (x2 minimum)	\$50,000 each
Social Worker	\$75,000
Chaplain	\$70,000

After Hours RN	\$95,000
Weekend RN	\$95,000
Medical Director (Contract)	\$75,000

As census grows, so does the clinical team. RN Case Managers are typically added for every 12 patients on service. Hospice Aides scale at roughly every 8 patients. The branch does not get to choose — it must staff to caseload or fall out of compliance.

Operational Staff — The Infrastructure Behind the Care

Care coordination requires people who never touch a patient directly but without whom the operation collapses.

Role	Annual Salary (Approx.)
Intake Coordinator	\$60,000
Secretary / Office Administrator	\$55,000

Sales and Marketing — The Rep Who May Not Know the Number

Role	Annual Salary + Benefits (Approx.)
Sales Rep / Marketer	\$115,000

This is you. Your salary is a fixed cost the branch pays regardless of your production. Every month you are employed, that cost hits the budget. The branch needs your production to justify that line item — and to cover every other line item on this list.

Per-Patient Daily Costs — What Each Admission Actually Costs to Serve

Beyond salaries, every patient in census generates a daily cost from clinical supplies, medications, and services consumed in the course of providing routine hospice care.

Cost Category	Daily Cost Per Patient (Approx.)
Pharmacy / Medications	\$22 - \$44
Durable Medical Equipment (DME)	\$10
Medical Supplies	\$10 - \$23
Clinical Travel	\$6
Other Patient-Related Costs	\$5
Total Daily Variable Cost	\$53 - \$88 per patient, per day

High-acuity patients — those with advanced cardiac disease, ALS, or complex pain management needs — can push daily costs significantly higher.

Monthly Overhead — The Cost of Running a Business

In addition to salaries and patient costs, every branch carries fixed operational overhead. Rent, utilities, liability insurance, billing software, compliance training, office supplies, and many other items exist before anyone walks in the door.

Typical monthly overhead for a small to mid-size hospice branch: \$35,000 - \$45,000 per month. Annualized, that is \$420,000 - \$540,000 in overhead before counting a single salary or serving a single patient.

PART TWO

Where the Revenue Comes From

Medicare pays hospice under a per diem model called Routine Home Care (RHC). For each day a patient is in hospice and receiving care at home, the branch receives a fixed daily rate from Medicare.

Payment Period	Daily Rate (Approx.)
Days 1-60 of a spell	\$208 - \$224 per patient per day
Days 61 and beyond	\$177 - \$209 per patient per day

After subtracting daily patient costs, the contribution margin — the amount each patient day contributes toward covering salaries and overhead — is roughly \$120 - \$155 per patient per day. That number is the engine. Every patient on census is generating that much daily toward covering the fixed costs of the branch.

PART THREE

The Break-Even Number — The Survival Line

Break-even ADC is the minimum Average Daily Census a branch must maintain for its revenue to exactly cover all of its costs — salaries, per-patient costs, and overhead combined. It is not a growth target. It is the survival line.

A typical hospice branch requires somewhere between 35 and 55 patients on census at all times just to break even — before generating a single dollar of profit.

Below break-even ADC, the branch loses money every single day. The losses are real, they compound, and at a certain point they threaten the financial viability of the entire operation. The gap between current census and break-even is not a gap in numbers. It is a gap in eligible patients who are not getting care — and a gap in financial health that threatens the branch's ability to keep its doors open.

PART FOUR

The Target Margin ADC — The Real Sales Goal

Break-even is not the mission. Break-even means the branch survived the month. Healthy hospice organizations operate at an operating margin of 12-18%. Target Margin ADC is the census level where the branch achieves its operating margin goal — not just survival, but sustainable, investable performance.

The difference between break-even ADC and target margin ADC is typically 15 to 30 additional patients on census. Every admission a rep converts, and every patient who stays on service longer than expected, moves the branch closer to that target.

PART FIVE

What This Means for You in the Field

When your manager says 'we need three more admissions this month,' they may be talking about keeping the branch above break-even. Three admissions at a 90-day average length of stay adds roughly 7-8 patients to the running ADC over a quarter — representing \$150,000 or more in annual contribution margin.

When a referral stalls and you let it go, the cost is not just a missed commission. It is a patient who did not get care, and a gap in census the branch must absorb financially while still paying every salary listed above.

When a patient is admitted with a short prognosis and stays only two weeks, the revenue barely covers the admission cost. The patients who drive branch sustainability are those who were referred early — when hospice could still provide months of meaningful support.

When you understand that every conversation you have in the field is connected to the financial engine of the branch, the work becomes less transactional and more purposeful. You are not just moving referrals. You are keeping the branch viable so it can continue to serve the community.

PART SIX

The Admissions Math — What You Actually Need to Produce

To maintain a given ADC, a branch must continuously replace patients who have died or discharged. The formula is straightforward:

$$\text{Monthly Admissions Needed} = (\text{Target ADC} \times 365) / \text{Average Length of Stay} / 12$$

For a branch targeting 60 ADC with a 90-day average length of stay, that is approximately 24 admissions per month — roughly 6 per week — just to hold census flat. If two marketers split that production, each rep needs to close approximately 12 admissions per month to keep the branch at census target.

That is not a soft goal. That is the math.

The Branch Profitability Tool — Built for This Conversation

The Spartan Coaching Branch Profitability Simulator exists because this education rarely happens in the field. It is a financial modeling tool that lets you — or your organization — input the actual variables of a hospice branch and see in real time what the numbers mean.

What you can model:

- Your current or projected ADC and what it means for annual profit and operating margin
- The full staffing cost structure as it scales with census
- Per-patient daily costs across low, base, and high-acuity patient populations
- Your branch's break-even ADC — the floor you must stay above
- Your target margin ADC — the number that represents healthy, investable performance
- How many monthly admissions are required to sustain your census goal
- How many marketers are needed to hit that admission target
- Cash runway — how long starting capital can absorb negative cash flow before break-even must be reached

Understanding the business you sell for makes you a better partner to your leadership team, a more credible advocate for your patients, and a more intentional professional in every conversation you have with a referral source.

Every patient below break-even is a financial loss. Every patient above target margin ADC is a contribution to the mission. But the real number — the one that matters most — is the number of eligible patients who received care earlier in their journey because a prepared, informed rep had the right conversation at the right time.

When you understand both sides of this equation — the cost structure and the care mission — you stop being a quota-chasing rep and start being a professional who understands exactly what their work means.

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Use the Branch Profitability Simulator to model your branch's specific numbers. Every input is customizable. Every output is actionable.